CASE STUDY

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"I never knew itching was bad for your skin"

What began as an ordinary itch on the nape of Amit Khanna's neck soon became a source of constant distress. As the irritation worsened, he scratched incessantly, leaving the skin thick, raised and lumpy. "I began to fear it might be a tumour," he recalls. The anxiety began affecting his work and relationships at home. Persuaded by his wife to seek medical help, he eventually consulted an oncologist and was diagnosed with eczema. Treatment has helped, but the affected area remains discoloured. "I never knew itching was bad for your skin," he says. Still self-conscious, Khanna wears high-collared shirts even during Delhi's sweltering summers.

the fungus," says Dr D.K. Sharma, senior consultant dermatologist at Kailash Hospital, Noida. "Used inappropriately, they can suppress the skin's local immune response, allow the infection to persist and alter its appearance. The rash may seem better for a while, only to return once the cream is stopped, sometimes making the eventual diagnosis more difficult." In a multicentre study published in the medical journal *Mycoses* in 2022, 17.31 per cent of the 41,421 patients screened across 13 dermatology centres in India reported fungal infection. Of these, 50.9 per cent reported misuse of topical corticosteroid creams.

Prolonged or inappropriate use of these creams can damage the skin itself. A 2021 Indian pharmacovigilance study published in the *Indian Journal of Pharmacology*, which examined 485 reports of adverse effects linked to steroid-containing creams, found that nearly 88 per cent of the preparations had been purchased without a prescription, most commonly for fungal infections. Skin thinning was reported in 50.1 per cent of adverse-event cases, stretch marks in 25.54 per cent and loss of pigmentation in 19.79 per cent. "Combination creams containing a steroid should not be used simply because a rash itches," says Dr D.K. Sharma. "The correct treatment depends on whether the cause is fungal, allergic, inflammatory or something else. Antifungal treatment, when required, also needs to be continued for the prescribed duration rather than stopped as soon as the itching settles."



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—DR D.M. MAHAJAN
Dermatologist, Apollo Hospitals, Delhi

Dermatologists are also beginning to get concerned about fungal infections that either do not respond as expected or keep returning after treatment. A multicentre Indian study published in *Mycoses* in 2020, involving 402 patients across eight locations, found that 71 per cent of their test sample isolates of dermatophytes—fungi that cause common skin infections such as ringworm, athlete's foot and jock itch—were resistant to terbinafine, a commonly used antifungal drug. "Repeated self-medication, incomplete treatment and indiscriminate use of steroid-antifungal combinations make management much more difficult," says Dr Bambroo. "If a fungal rash keeps coming back, it needs proper diagnosis rather than simply another tube of cream."

NEW TREATMENTS

Diagnoses as well as treatments are now becoming more targeted, focusing on the exact cause rather than merely suppressing symptoms (see *What's Changing in Skin Treatment*). "Skin is an organ, not simply something we look after for cosmetic reasons," says Dr D.M. Mahajan, a dermatologist with Apollo Hospitals, Delhi. "There is no single medicine for itching because there is no single cause." So, dry or eczema-prone skin may need moisturisation to repair its protective barrier; an allergy may require identifying and avoiding the specific trigger; fungal infections need appropriate antifungal treatment; and persistent unexplained itching may sometimes require investigation beyond the skin.

Diagnosis itself begins with a skin examination. The doctor asks when and where the itching began, what improves or worsens it, which products or medicines the patient uses, and whether a visible rash or other clue is present. If the cause remains unclear, blood tests may be ordered to check for allergies to food, pollen, dust or pet dander. As Dr Arjun Dang of Dr Dang's Path Labs notes, "Diagnostics has become much more focused on identifying the exact trigger rather than simply labelling a rash as 'allergic'." Patch testing can pinpoint substances in cosmetics, perfumes, hair dyes, nickel or preservatives that trigger a reaction, while specialised photopatch testing can show whether a reaction occurs only after exposure to sunlight. Multidimensional itch scales assess severity, duration, progression, disability and distribution, helping track chronic pruritus associated with liver, kidney and skin conditions. For difficult-to-treat fungal infections, laboratory tests can determine whether the organism is resistant to specific antifungal drugs. Some centres also offer genetic testing for mutations linked to antifungal resistance, helping explain why standard treatment has failed. Scientists at Harvard Medical School, for instance, have shown that the common skin bacterium *Staphylococcus aureus* can cause itching by acting directly on nerve cells.

Treatments, on the other hand, have moved away from traditional anti-hista-